

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Ibuprofen and Amoxicillin for Dental Pain for the treatment of Dental Pain
Bridging Care

Summary of NICE / NICE CKS Guidelines for Dental Pain Bridging Care

Dental pain management and assessment

Acute dental pain often presents with difficulty accessing emergency dental services. This PGD provides bridging pain relief and targeted antibiotic therapy when indicated, as an interim measure until urgent dental assessment can be arranged.

- Dental pain assessment: localise source, severity, duration, associated symptoms (swelling, fever, lymphadenopathy)
- Determine urgency: severe systemic infection (facial swelling, fever, malaise) requires urgent referral; localized dental pain may be managed in primary care pending dental appointment
- Antibiotics indicated only if signs of systemic infection or spreading infection present

Ibuprofen for pain and inflammation management

Ibuprofen is an NSAID providing effective analgesia and anti-inflammatory action for dental pain. Dosing should be optimized to minimize duration of use pending dental intervention.

- Effective anti-inflammatory action helps reduce swelling and associated pain
- Short-term use (3 days) carries low risk; should be taken with food to minimize GI risk
- Patient must arrange urgent dental appointment; this is bridging treatment only

Amoxicillin for dental abscess and spreading infection

Amoxicillin is indicated only when there is clinical evidence of systemic infection or spreading infection from dental source. Signs include fever, lymphadenopathy, facial swelling, or malaise.

- Beta-lactam antibiotic with broad spectrum activity against oral streptococci and anaerobes
- Use restricted to cases with clear signs of systemic or spreading infection
- 5-day course standard for acute dental infection pending definitive dental management

Patient Group Direction

for the supply/administration of

Ibuprofen 400mg tablets

for the treatment of

Dental Pain Bridging Care

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	- Pharmacist registered and practicing with the GPhC - Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute dental pain and associated inflammation, to provide bridging analgesia pending urgent dental appointment
Inclusion criteria	Adult patients (18+ years) presenting with acute dental pain Pain localized to tooth or dental structure Able to swallow tablets Willing to arrange urgent dental appointment
Exclusion criteria	Active peptic ulcer disease or history of GI bleeding/perforation Severe renal impairment (eGFR <30) Severe hepatic impairment Severe cardiac failure or uncontrolled hypertension Third trimester pregnancy Hypersensitivity to ibuprofen or aspirin/NSAID allergy Concurrent use of anticoagulants (relative contraindication - evaluate benefit-risk) Acute severe systemic infection with fever >38.5°C (may need urgent medical assessment rather than bridging care)
Cautions	History of asthma (NSAIDs may precipitate bronchospasm)

	Cardiovascular disease or significant cardiovascular risk factors Elderly patients (>65 years) - increased risk of GI, renal, and cardiovascular adverse effects Mild to moderate renal impairment - monitor for deterioration Mild to moderate hepatic impairment - use lowest effective dose for shortest duration Concurrent corticosteroids - increased GI risk, counsel on administration with food Concurrent SSRIs or other anticoagulants - increased GI bleeding risk Hypertension - NSAIDs may reduce blood pressure control; monitor BP
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Ibuprofen 400mg tablets
Legal category	POM (Pharmacy supply under PGD)
Storage	Store below 25°C. Keep in original container. Do not use after expiry date.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	400mg three times daily (TDS) with food Maximum 1200mg daily (three 400mg tablets) Use for up to 3 days only pending dental appointment
Quantity to be administered and/or supplied	9 tablets (3 tablets daily x 3 days)
Maximum or minimum treatment period	Up to 3 days. Patient must arrange urgent dental appointment within this timeframe.
Adverse effects	Common: dyspepsia, nausea, gastric discomfort (minimize by taking with food/milk) Uncommon: epigastric pain, gastric ulceration with prolonged use Rare: GI bleeding, perforation (higher risk with prolonged use or in elderly) Rare: hypersensitivity reactions including rash or bronchospasm Rare: renal impairment or acute kidney injury (especially with

	dehydration or pre-existing renal disease) Very rare: hepatotoxicity
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply patient information leaflets (PILs) for ibuprofen and amoxicillin.
Follow-up advice to be given to patient or carer	THIS IS BRIDGING TREATMENT ONLY - You MUST arrange an urgent dental appointment within 24-48 hours, ideally with your regular dentist or via NHS emergency dental services. Contact NHS 111 if you cannot reach your dentist. Take ibuprofen with food or milk to reduce stomach upset. Do not take for more than 3 days. If antibiotics (amoxicillin) have been prescribed, take the full course (5 days) as directed, even if you feel better. Amoxicillin may cause diarrhoea; maintain good fluid intake and hand hygiene. Report allergic symptoms immediately: rash, swelling of lips/tongue, difficulty breathing, or wheezing - seek urgent medical attention (call 999 if severe). Pain should improve within 24-48 hours. If pain worsens despite treatment, or if fever develops/worsens, contact your GP or call NHS

Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Amoxicillin 500mg capsules

for the treatment of

Dental Pain Bridging Care

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute dental abscess or spreading infection confirmed by signs of systemic infection (fever, lymphadenopathy, facial swelling)
Inclusion criteria	Clinical signs of systemic or spreading infection: fever ($\geq 38^{\circ}\text{C}$), regional lymphadenopathy, facial swelling, or malaise Presumed bacterial source from dental focus Willing to arrange urgent dental appointment within 24-48 hours Able to swallow capsules
Exclusion criteria	Penicillin or amoxicillin allergy (including anaphylaxis, Stevens-Johnson syndrome, or severe rash history) Cephalosporin allergy (consider cross-reactivity risk; generally low) Glandular fever (amoxicillin associated with rash in infectious

	mononucleosis) Acute severe systemic illness suggesting need for urgent medical/hospital assessment (sepsis, airway involvement, severe malaise) - refer to A&E Pregnant women: amoxicillin is safe in pregnancy; should be used if clinically indicated Breastfeeding: amoxicillin is safe; passes into breast milk at low levels
Cautions	History of allergic reactions to any beta-lactam: assess cross-reactivity risk before prescribing Diarrhoea: amoxicillin-associated diarrhoea is common; counsel patient on fluid intake and hygiene Renal impairment: dose adjustment may be needed for significant impairment; refer to BNF Hepatic impairment: use with caution; monitor for signs of hepatotoxicity
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Amoxicillin 500mg capsules
Legal category	POM
Storage	Store below 25°C. Keep in original container. Do not use after expiry date.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	500mg three times daily (TDS) One capsule every 8 hours Take with or without food
Quantity to be administered and/or supplied	15 capsules (5 per day x 3 days; includes one extra as buffer)
Maximum or minimum treatment period	5 days only for acute dental infection pending urgent dental appointment
Adverse effects	Very common: diarrhoea (mild to moderate, usually self-limiting) Common: nausea, rash (non-allergic, maculopapular) Uncommon: vomiting, abdominal discomfort, oral thrush (candidiasis)

	Rare: allergic reactions (rash, urticaria, pruritus) Very rare: anaphylaxis (immediate management required), Stevens-Johnson syndrome, hepatitis Very rare: Clostridioides difficile-associated colitis (risk higher with prolonged use)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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	999 if severe). Pain should improve within 24-48 hours. If pain worsens despite treatment, or if fever develops/worsens, contact your GP or call NHS 111. If facial swelling, difficulty swallowing, difficulty breathing, or severe malaise develop at any point, seek immediate medical attention (call 999 or attend A&E). Avoid alcohol while taking ibuprofen and amoxicillin. Do not exceed 3 days of ibuprofen use without dental review. Once dental appointment is arranged, inform the dentist which medications you have taken.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Dental Pain Bridging Care
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

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